

Access guide — urothelial-mets-her2-discordant-kndl

How to access each therapy: trial contacts + manufacturer medical-info

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How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

Summary

#	Intervention	Modality	Access status	Regulatory	Recommended first action
1	enfortumab vedotin (Padcev)	ADC	Standard of care	FDA-approved for locally advanced/metastatic urothelial carcinoma (with pembrolizumab and as monotherapy)	Keep in reserve; rechallenge only if the CIPN resolves substantially and Nectin-4 expression is reconfirmed on contemporary tissue or imaging.
2	pembrolizumab (TMB-high, tumor-agnostic)	monoclonal_antibody	Standard of care	FDA-approved for urothelial carcinoma and tumor-agnostic TMB-H (≥ 10 mut/Mb)	Keep the hold/restart decision tied to the next therapy choice; several trial rows allow pembrolizumab combination arms.
3	trastuzumab deruxtecan (T-DXd)	ADC	Standard of care	FDA-approved, tumor-agnostic for HER2-positive (IHC 3+) solid tumors after prior systemic therapy (accelerated approval 2024); multiple full approvals in breast and gastric cancer	Retrieve the 2026-08 repeat validated HER2 IHC tier; it decides continue versus stop.
4	EZH2 inhibition (tazemetostat) for ARID1A-mutant disease	small_molecule	Off-label use	FDA-approved (epithelioid sarcoma, follicular lymphoma); off-label and hypothesis-grade here	No action; revisit only if new ARID1A-EZH2 clinical data emerge.

5	N-803 / nogapendekin alfa inbakicept (Anktiva)	other	Off-label use + Compassionate use	FDA-approved (2024) with BCG for BCG-unresponsive NMIBC with CIS; off-label in metastatic urothelial carcinoma; FDA-authorized expanded-access protocol exists for a lymphopenia indication	Have the treating physician call ImmunityBio Medical Affairs (1-877-265-8482) to formalize the single-patient IND with drug supply.
6	PARP inhibition (with temozolomide) for ARID1A-mutant disease	small_molecule	Off-label use	Class FDA-approved in BRCA/HRR-selected indications; off-label and here	Hold unless the full NGS report reveals an HRR alteration; then reassess against TAPUR-style basket options.
7	datopotamab deruxtecan (Dato-DXd)	ADC	Off-label use + Clinical trial	FDA-approved in HR+/-HER2- breast cancer and EGFR-mutated NSCLC; investigational in urothelial carcinoma	Resolve the ILD read first; a deruxtecan-class ADC is hard to justify while a fibrosis question is open.
8	nivolumab plus ipilimumab (checkpoint re-intensification)	monoclonal_antibody	Off-label use + Clinical trial	Both FDA-approved (multiple indications); the doublet is off-label in urothelial carcinoma	Get an expert pathology re-read quantifying plasmacytoid and giant-cell percentages; it decides trial versus off-label.
9	sacituzumab govitecan (Trodelvy)	ADC	Off-label use + Clinical trial	FDA-approved in triple-negative and HR+/-HER2- breast cancer; urothelial accelerated approval withdrawn 2024	Have the treating team document formal platinum ineligibility (cisplatin and carboplatin criteria) in the chart.
10	trastuzumab plus paclitaxel	monoclonal_antibody	Off-label use	Both components FDA-approved; combination off-label for urothelial carcinoma	Hold this row unless the CIPN resolves to grade 1 and the chemotherapy-free options are exhausted.
11	trastuzumab plus pertuzumab	monoclonal_antibody	Off-label use + Clinical trial	Both FDA-approved (breast, gastric for trastuzumab); off-label in urothelial carcinoma	Retrieve the full tissue NGS report and check for ERBB2 amplification.

12	zanidatamab (HER2 biparatopic bispecific antibody)	bispecific_other	Off-label use + Clinical trial	FDA accelerated approval (2024-11) for previously treated HER2-positive (IHC 3+) biliary tract cancer; investigational in urothelial carcinoma	Wait for the 2026-08 repeat HER2 IHC tier; only a confirmed 3+ supports this row.
13	A2B694 (logic-gated Tmod mesothelin CAR-T)	CAR-T	Clinical trial	Investigational; no approval anywhere	Run the HLA-A*02 LOH computation on the banked WES data; it costs nothing and is the gating fact.
14	ART0380 (ATR inhibitor)	small_molecule	Clinical trial	Investigational; no approval anywhere	Retrieve the full NGS report first; an HRD or ATM finding changes which cohort applies.
15	ASP2998 (TROP2-directed STING agonist conjugate)	ADC	Clinical trial	Investigational; no approval anywhere	Contact the Hackensack site about escalation-cohort openings for urothelial carcinoma.
16	AVZO-103 (Nectin-4 x TROP2 bispecific ADC)	ADC	Clinical trial	Investigational; FDA Fast Track for post-EV urothelial carcinoma	Get a pulmonology adjudication of the 2026-03 versus 2026-06 chest CT reads documented in the chart first.
17	CAdVEC oncolytic adenovirus plus autologous HER2 CAR virus-specific T cells	CAR-T	Clinical trial	Investigational; no approval anywhere	Ask the Baylor team whether a post-SBRT para-aortic node is injectable on their protocol before anything else.
18	CLSP-1025 (TP53 R175H x HLA-A*02:01 T-cell engager)	bispecific_other	Clinical trial	Investigational; no approval anywhere	Confirm R175H in tissue: check whether the 2023 NGS panel's TP53 call specifies R175H before relying on the 0.09% plasma read.
19	CRB-701 (site-specific Nectin-4 ADC, MMAE payload)	ADC	Clinical trial	Investigational; no approval anywhere	Only pursue if a formal CIPN assessment grades below 2; otherwise the MMAE rows stay closed.
20	CT-202 (Nectin-4-directed bispecific antibody)	bispecific_other	Clinical trial	Investigational; no approval anywhere	Email the sponsor about US site plans; enrolling a US patient in Australia for a phase 1 is a last resort.
21	CT-95 (mesothelin x CD3 bispecific antibody)	bispecific_other	Clinical trial	Investigational; no approval anywhere	Order a validated mesothelin IHC on existing tissue first; mind the 5-10% tumor-content caveat on the nodal cores.
22	DS-3939a (TA-MUC1-directed ADC)	ADC	Clinical trial	Investigational; no approval anywhere	Email CTRinfo@dsi.com to ask whether urothelial carcinoma can screen and what MUC1 assay gates entry.

23	E303 / SBE303 (Nectin-4 ADC)	ADC	Clinical trial	Investigational; no approval anywhere	Email the sponsor to ask what the payload is; that answer decides whether this row is live.
24	EB-DT-NK-UC101 (Nectin-4/HER2 dual-target CAR-NK)	CAR-NK	Clinical trial + Not yet accessible	Investigational; no approval anywhere	Track for a US or ex-China site opening; no outreach warranted now.
25	FAP-targeted radioligand therapy (FAP-2286 / FAPI class)	radioligand	Clinical trial	Investigational; no approval anywhere	Retrieve the 2026-06 repeat FAP imaging read; it decides whether this axis is real.
26	FT825 / ONO-8250 (off-the-shelf iPSC-derived HER2 CAR-T)	CAR-T	Clinical trial	Investigational; no approval anywhere	Send the trial contact the HER2 history and the 2026-08 re-stain when it lands; ask which dose cohort is open.
27	IMA203 / IMA203CD8 (PRAME-directed TCR-T)	CAR-T	Clinical trial	Investigational; no approval anywhere	Retrieve the 2026-08 PRAME imaging read; if it suggests expression, contact Immatics about rare-cancer cohort screening.
28	IMC-P115C (half-life-extended PRAME ImmTAC)	bispecific_other	Clinical trial	Investigational; no approval anywhere	Ask Immunocore medical information whether a US site is planned and whether tebentafusp-style expanded access is contemplated for PRAME.
29	LCB84 (TROP2-directed ADC)	ADC	Clinical trial	Investigational; no approval anywhere	Ask the sponsor whether an escalation slot without mandatory biopsies is open before spending a screening visit.
30	LY4052031 (Nectin-4 ADC, camptothecin payload)	ADC	Clinical trial	Investigational; no approval anywhere	Call the Lilly trials line or have the treating team contact the nearest site (MSK or Mount Sinai) to confirm B2 slot availability.
31	NEOK002 (EGFR x MUC1 bispecific ADC)	ADC	Clinical trial	Investigational; no approval anywhere	Park until an intervening non-topo-I regimen exists; then revisit with a validated MUC1 stain and the ILD read.
32	NT-175 (autologous TCR-T targeting TP53 R175H)	CAR-T	Clinical trial	Investigational; no approval anywhere	Call the AstraZeneca study information center to ask about slot and manufacturing timelines at the New York or Boston site.
33	SynKIR-110 (mesothelin KIR-CAR T cells)	CAR-T	Clinical trial	Investigational; no approval anywhere	No action; watch for a histology expansion.

34	TNhYP218 (anti-mesothelin TNaive/SCM CAR T cells)	CAR-T	Clinical trial	Investigational; no approval anywhere	Send pathology and the MXIF report to the NCI contacts and ask whether they will run their mesothelin assay on existing tissue.
35	XYA02 (MUC1-directed agent)	other	Clinical trial + Not yet accessible	Investigational; no approval anywhere	Track for opening and for construct disclosure; no outreach warranted now.
36	(Nectin-4 actinium-225 radioligand)	radioligand	Clinical trial	Investigational; no approval anywhere	Email the sponsor inquiry line to ask about screening at Mount Sinai and current dose-escalation stage.
37	ceralasertib (AZD6738, ATR inhibitor)	small_molecule	Clinical trial	Investigational; no approval anywhere	Route ATR-axis interest through the ART0380 row; no ceralasertib protocol currently fits this case.
38	disitamab vedotin (RC48-ADC)	ADC	Clinical trial + Not yet accessible	NMPA-approved (China) for urothelial and gastric cancer; investigational in the US (no FDA approval)	No action; revisit only if a next-generation disitamab protocol without the prior-HER2/prior-MMAE exclusions opens.
39	neoantigen-selected TIL with aldesleukin (NCI Surgery Branch)	other	Clinical trial	Investigational; no approval anywhere	Call the Surgery Branch recruitment line with imaging and ask whether any current or anticipated lesion supports TIL harvest.
40	personalized neoantigen vaccines (peptide + mRNA programs)	vaccine	Clinical trial	Investigational; administered under the programs' own protocols	Complete the 2026-09 mRNA dose and the post-series EliSpot; that readout drives the whole vaccine/ACT branch.
41	sacituzumab tirumotecan (sac-TMT, MK-2870)	ADC	Clinical trial	Investigational in the US (approved in China for some indications); no FDA approval	No enrollment path as written; ask MSK whether other sac-TMT urothelial protocols without the platinum requirement are planned.
42	tuvusertib (M1774, ATR inhibitor) with avelumab	small_molecule	Clinical trial	Tuvusertib investigational; avelumab FDA-approved (urothelial maintenance among others)	Ask the DFCI investigator whether an ARID1A basket or urothelial arm is planned; otherwise no action.
43	vepugratinib (LOXO-435, FGFR3 inhibitor)	small_molecule	Clinical trial	Investigational; no approval anywhere	Retrieve the full tissue NGS report and read out FGFR3; this is already the case's top data-retrieval item.

44	brenetafusp (IMC-F106C, PRAME ImmTAC)	bispecific_other	Not yet accessible	Investigational; no approval anywhere	If PRAME confirms positive, ask Immunocore medical information about expanded access to brenetafusp for an HLA-matched, PRAME-positive off-melanoma patient.
45	adenoviral vaccine	vaccine	Unavailable	Investigational; program dormant	None.
46	ETx-22 (Nectin-4 ADC, exatecan payload, preclinical)	ADC	Unavailable	Preclinical; no clinical trial registered	Pursue LY4052031 instead; it is the enrolling embodiment of the same idea.
47	M7A-DC / M7GA-DC (mesothelin DARPIn-MMAE conjugates, preclinical)	other	Unavailable	Preclinical; no clinical trial registered	None.
48	MUC1-C inhibition (GO-203 class, preclinical for this use)	small_molecule	Unavailable	Investigational; no enrolling solid-tumor trial	None.
49	Nectin-4-directed CAR T cells (preclinical)	CAR-T	Unavailable	Preclinical; no IND on record	None; revisit if a first-in-human Nectin-4 CAR program posts.
50	Nectin-4-targeted near-infrared (preclinical)	other	Unavailable	Preclinical; no clinical trial registered	None.
51	R7059-DXd (epitope-distinct TROP2 ADC, preclinical)	ADC	Unavailable	Preclinical; no clinical trial registered	None.
52	anetumab ravtansine (mesothelin ADC)	ADC	Unavailable	Investigational; sponsor development discontinued	None.
53	berzosertib (M6620, ATR inhibitor)	small_molecule	Unavailable	Investigational; sponsor development deprioritized, no recruiting trial	None; the ATR door is ART0380.
54	gatipotuzumab (anti-TA-MUC1 antibody)	monoclonal_antibody	Unavailable	Investigational; no active development	None; pursue the DS-3939a row instead.
55	gavocabtagene autoleucel (gavo-cel)	CAR-T	Unavailable	Investigational; development deprioritized after the 2023 TCR2/Adaptimmune merger	None; the open mesothelin cell-therapy doors are TNhYP218 and, conditionally, A2B694.

56	huCART-meso (fully human anti-mesothelin CAR T cells)	CAR-T	Unavailable	Investigational; academic program with no recruiting protocol	None; route mesothelin cell-therapy interest to the TNhYP218 row.
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Standard of care (3)

1. enfortumab vedotin (Padcev)

Aliases: EV, Padcev, enfortumab vedotin-ejfv

Modality: ADC · **Regulatory:** FDA-approved for locally advanced/metastatic urothelial carcinoma (with pembrolizumab and as monotherapy) · **Guidelines:** NCCN category 1 (with pembrolizumab) for first-line metastatic urothelial carcinoma · **Verified:** 2026-08-20

On-label and freely prescribable in this indication, and the patient already took it for about 25 months to a complete response before neuropathy forced discontinuation. Rechallenge is a real, documented practice after toxicity interruption, but the grade 2 CIPN with an active management project makes further MMAE exposure the thing the preferences file most clearly vetoes. Access is trivial; the constraint is entirely toxicity.

Next steps

1. Keep in reserve; rechallenge only if the CIPN resolves substantially and Nectin-4 expression is reconfirmed on contemporary tissue or imaging.
2. Prefer the non-MMAE Nectin-4 rows (LY4052031, AKY-1189) for re-targeting the axis.

Manufacturer / sponsor contact

- **Company:** Astellas Pharma (with Pfizer/Seagen for enfortumab vedotin)
- **Med info phone:** 1-800-727-7003
- **Product info:** <https://astellasmedinfo.com/>
- **Notes:** Astellas US medical information line.

Payer / coverage: On-label; no access barrier. Astellas Pharma Support Solutions handles copay and access casework.

2. pembrolizumab (TMB-high, tumor-agnostic)

Aliases: Keytruda, MK-3475

Modality: monoclonal_antibody · **Regulatory:** FDA-approved for urothelial carcinoma and tumor-agnostic TMB-H (≥ 10 mut/Mb) · **Guidelines:** NCCN-listed across urothelial settings · **Verified:** 2026-08-20

On-label twice over (urothelial carcinoma and the TMB ≥ 10 tumor-agnostic approval) and the patient has been on it for years, so there is nothing to obtain. Progression occurred on pembrolizumab maintenance, which is why re-challenge alone is weakly supported and the live question is intensification on top of it, handled in the nivolumab-ipilimumab and N-803 rows. Currently on hold since 2026-07.

Next steps

- 1. Keep the hold/restart decision tied to the next therapy choice; several trial rows allow pembrolizumab combination arms.

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme
- **Notes:** Trial-site inquiries go through the registry contact on each study record; no separate medical-information line verified this pass.

Payer / coverage: On-label; no access barrier. The cross-assay TMB caveat (institutional panel versus FoundationOne CDx) matters for documentation if the TMB-H label is the coverage basis.

3. trastuzumab deruxtecan (T-DXd)

Aliases: T-DXd, DS-8201a, fam-trastuzumab deruxtecan-nxki, Enhertu

Modality: ADC · **Regulatory:** FDA-approved, tumor-agnostic for unresectable/metastatic HER2-positive (IHC 3+) solid tumors after prior systemic therapy (accelerated approval 2024); multiple full approvals in breast and gastric cancer · **Guidelines:** NCCN lists T-DXd for HER2-overexpressing urothelial carcinoma post-platinum/post-EV settings per the tumor-agnostic approval · **Verified:** 2026-08-20

T-DXd is the current therapy and is on-label for this patient under the tumor-agnostic HER2 IHC 3+ approval, so access is not the problem. The live questions are whether to keep giving it: the 2026-08 repeat HER2 IHC has to reconcile the discordant weak MXIF signal, the severe nausea and bloating after dose 2 need a management plan or a stop decision, and the conflicting 2026-03 chest CT read has to be resolved because drug-related ILD is the toxicity that kills patients on this drug.

Next steps

- 1. Retrieve the 2026-08 repeat validated HER2 IHC tier; it decides continue versus stop.
- 2. Get a dedicated chest CT or pulmonology read to reconcile the 2026-03 versus 2026-06 ILD discrepancy before the next dose.
- 3. If continuing, set an antiemetic plan (olanzapine-based) and a dose-reduction threshold for the GI toxicity.
- 4. Use the next Signatera and NeXT Personal draws as the response arbiter given the post-SBRT measurable-disease ambiguity.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04422309		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Daiichi Sankyo (co-developed with AstraZeneca)
- **Med info phone:** 1-877-437-7763

- **Product info:** <https://dsimedinfo.com/>
- **Notes:** US medical information line, Monday-Friday 9am-6pm ET.

Payer / coverage: Covered on-label under the tumor-agnostic approval when current IHC 3+ documentation is in the chart; a downgrade to HER2-low on the repeat stain would move continued use off-label and invite prior-authorization pushback.

Off-label use (9)

4. EZH2 inhibition (tazemetostat) for ARID1A-mutant disease

Aliases: tazemetostat, Tazverik

Modality: small_molecule · **Regulatory:** FDA-approved (epithelioid sarcoma, follicular lymphoma); off-label and hypothesis-grade here · **Verified:** 2026-08-20

Tazemetostat is approved in epithelioid sarcoma and follicular lymphoma, so off-label use is possible on paper, but the dedicated ARID1A-mutant solid-tumor study was terminated and the rationale for this case remains preclinical. Logged as a closed loop rather than a live option.

Next steps

1. No action; revisit only if new ARID1A-EZH2 clinical data emerge.

Payer / coverage: Off-label coverage unlikely without trial-level evidence; the ARID1A basket that would have generated it (NCT05023655) was terminated.

5. N-803 / nogapendekin alfa inbakicept (Anktiva)

Aliases: N-803, ALT-803, Anktiva, nogapendekin alfa inbakicept-pmln

Modality: other · **Regulatory:** FDA-approved (2024) with BCG for BCG-unresponsive NMIBC with CIS; off-label in metastatic urothelial carcinoma; FDA-authorized expanded-access protocol exists for a lymphopenia indication · **Guidelines:** NCCN-listed for BCG-unresponsive NMIBC; off-guideline for metastatic disease · **Verified:** 2026-08-20

Approved for BCG-unresponsive non-muscle-invasive bladder cancer, so off-label systemic use in metastatic disease is prescribable, and ImmunityBio operates expanded-access machinery including an FDA-authorized ANKTIVA protocol and a posted compassionate-use page. The single-patient IND the care team is already discussing for NK cells plus N-803 is the same pathway this row supports; the QUILT trial that tested the exact concept closed accrual, so no enrollment route exists.

Next steps

1. Have the treating physician call ImmunityBio Medical Affairs (1-877-265-8482) to formalize the single-patient

IND with drug supply.

2. Reference the posted expanded-access page and the existing FDA EAP as precedent in the request.
3. Coordinate with the PD-1 hold decision; the QUILT design paired N-803 with continued checkpoint blockade.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03228667		active not recruiting	no	—

Manufacturer / sponsor contact

- **Company:** ImmunityBio
- **Med info phone:** 1-877-265-8482
- **Compassionate / expanded access:** <https://immunitybio.com/expanded-access/>
- **Notes:** 1-877-ANKTIVA reaches Medical Affairs; the company posts an expanded-access page and has an FDA-authorized ANKTIVA expanded-access protocol (lymphopenia indication).

Payer / coverage: Off-label systemic use in metastatic disease is an uphill coverage case; the single-patient IND route with sponsor-supplied drug avoids the payer entirely if ImmunityBio agrees.

6. PARP inhibition (with temozolomide) for ARID1A-mutant disease

Aliases: olaparib, niraparib, talazoparib

Modality: small_molecule · **Regulatory:** Class FDA-approved in BRCA/HRR-selected indications; off-label and biomarker-unsupported here · **Verified:** 2026-08-20

PARP inhibitors are approved drugs, so off-label prescription is mechanically possible, but the ARID1A rationale is preclinical, urothelial carcinoma is outside every PARP label, and no germline or somatic HRR finding currently supports it. Coverage would be a fight and the biology does not yet earn one. The pending NGS retrieval is the only thing that could move this row.

Next steps

1. Hold unless the full NGS report reveals an HRR alteration; then reassess against TAPUR-style basket options.

Payer / coverage: Expect denial without an HRR biomarker; no PARP label covers urothelial carcinoma.

7. datopotamab deruxtecan (Dato-DXd)

Aliases: Dato-DXd, DS-1062, Datroway

Modality: ADC · **Regulatory:** FDA-approved in HR+/HER2- breast cancer and EGFR-mutated NSCLC; investigational in urothelial carcinoma · **Verified:** 2026-08-20

Approved in breast and lung cancer, so off-label prescription for urothelial carcinoma is possible, and a urothelial

substudy exists inside an AstraZeneca platform with loose line requirements. Two collisions to clear: the platform requires a measurable lesion that was not irradiated, which the SBRT-treated node may fail, and Dato-DXd carries the same deruxtecan ILD signal already at issue on T-DXd, so the unresolved chest CT read cuts against this whole payload class.

Next steps

1. Resolve the ILD read first; a deruxtecan-class ADC is hard to justify while a fibrosis question is open.
2. Ask the AstraZeneca information center whether the urothelial substudy is enrolling and whether any new measurable lesion exists outside the SBRT field.
3. Retrieve the validated TROP2 IHC tier for context, noting the platform does not gate on it.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05429211		recruiting	unconfirmed	AstraZeneca Clinical Study Information Center; information.center@astrazeneca.com ; 1-877-240-9479

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com
- **Notes:** AstraZeneca Clinical Study Information Center, as published on the registry records.

Payer / coverage: Off-label Dato-DXd in urothelial carcinoma will face prior-authorization resistance; on-protocol supply through the substudy avoids it.

8. nivolumab plus ipilimumab (checkpoint re-intensification)

Aliases: Opdivo, Yervoy, nivo/ipi

Modality: monoclonal_antibody · **Regulatory:** Both FDA-approved (multiple indications); the doublet is off-label in urothelial carcinoma · **Guidelines:** Off-guideline for urothelial carcinoma; NCCN does not currently list the doublet in this setting · **Verified:** 2026-08-20

Both antibodies are approved and the doublet is prescribable off-label now, which is what the care team is already exploring; payer friction and the 2025 pancreatitis history are the constraints, not supply. The registered route is the ICONIC/Alliance trial with cabozantinib, whose variant-histology cohorts want roughly 50% plasmacytoid or giant-cell composition that this tumor's focal pattern may not meet without a central re-read. Off-label dosing lets the team pick the gentler ipilimumab schedule the boost data support.

Next steps

1. Get an expert pathology re-read quantifying plasmacytoid and giant-cell percentages; it decides trial versus off-label.

2. If off-label, agree on a pancreatitis surveillance plan up front given the 2025 episode; lipase monitoring is already q3-4wk.
3. File prior authorization with the TMB 12.4, the deep 1L response and the progression-on-maintenance story.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03826382		recruiting	unconfirmed	—

Manufacturer / sponsor contact

- **Company:** Bristol Myers Squibb
- **Med info phone:** 1-800-861-0048
- **Notes:** BMS Access Support line (coverage and access, Monday-Friday 8am-8pm ET); route clinical questions through the prescriber portal at bmsmedinfo.com.

Payer / coverage: Off-label nivo/ipi in urothelial carcinoma needs prior authorization with the TMB and prior-response narrative attached; BMS Access Support (1-800-861-0048) runs coverage casework, and the trial route supplies drug on-protocol if a variant cohort fits.

9. sacituzumab govitecan (Trodelvy)

Aliases: IMMU-132, Trodelvy, sacituzumab govitecan-hziy

Modality: ADC · **Regulatory:** FDA-approved in triple-negative and HR+/HER2- breast cancer; urothelial accelerated approval withdrawn 2024 · **Guidelines:** Removed from NCCN urothelial recommendations after the withdrawal · **Verified:** 2026-08-20

The urothelial accelerated approval was withdrawn in 2024 after TROPiCS-04, so in this indication the drug is trial-only, and TROPY-U-01 cohort 2 happens to be the one registered cohort in the post-EV landscape built for a patient who never had platinum: it takes platinum-ineligible patients who progressed after first-line anti-PD-(L)1. The breast approvals still stand, so off-label prescription is technically possible, but payers will lean on the withdrawn UC indication. The team would need to document platinum ineligibility to enroll.

Next steps

1. Have the treating team document formal platinum ineligibility (cisplatin and carboplatin criteria) in the chart.
2. Call the Gilead trials line to confirm cohort 2 enrollment status and the nearest open Northeast site.
3. Retrieve the validated 2026-08 TROP2 IHC tier; low expression does not bar entry but calibrates expectations.
4. Sequence against the T-DXd washout; both payloads are topoisomerase I inhibitors and the site will ask about cumulative exposure.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** Gilead Sciences
- **Med info phone:** 1-833-445-3230
- **Med info email:** GileadClinicalTrials@gilead.com
- **Product info:** <https://www.askgileadmedical.com/>
- **Notes:** Medical information line 1-833-GILEAD-0; the email is the sponsor's clinical-trials contact from the registry record.

Payer / coverage: Off-label use in urothelial carcinoma after a withdrawn indication is a hard prior-authorization case; enrollment supplies drug on-protocol and is the cleaner route.

10. trastuzumab plus paclitaxel

Aliases: trastuzumab-pkrb, Herzuma, paclitaxel

Modality: monoclonal_antibody · **Regulatory:** Both components FDA-approved; combination off-label for urothelial carcinoma · **Guidelines:** Off-guideline for urothelial carcinoma · **Verified:** 2026-08-20

Both drugs are approved and any oncologist can give the combination off-label tomorrow, but the paclitaxel half runs straight into the established grade 2 CIPN from 25 months of enfortumab vedotin, and the case file treats severe neuropathy as a veto. Accessible, not advisable; the trastuzumab-pertuzumab row covers the same axis without a neurotoxic partner.

Next steps

1. Hold this row unless the CIPN resolves to grade 1 and the chemotherapy-free options are exhausted.

Payer / coverage: Generic paclitaxel and biosimilar trastuzumab make this the least expensive HER2 combination if it were ever wanted.

Notes: Included because the researcher file carries a supporting study; the neuropathy veto is the operative fact.

11. trastuzumab plus pertuzumab

Aliases: TAPUR, Herceptin, Perjeta, trastuzumab biosimilars

Modality: monoclonal_antibody · **Regulatory:** Both FDA-approved (breast, gastric for trastuzumab); off-label in urothelial carcinoma · **Guidelines:** Off-guideline for urothelial carcinoma; TAPUR provides the protocolized route · **Verified:** 2026-08-20

The TAPUR basket is the most enrollable trial in this dossier: pragmatic entry criteria, ECOG 0-2, sites across

Connecticut and Maine among 181 locations, and a HER2 arm keyed to ERBB2 amplification or overexpression. Whether this patient's arm is open turns on the pending full NGS report, since the 2023 panel counted copy-number alterations without naming them. Both antibodies are approved with cheap biosimilars, so plain off-label use is also on the table, with modest expected activity in a T-DXd-exposed tumor.

Next steps

- 1. Retrieve the full tissue NGS report and check for ERBB2 amplification.
- 2. Email tapur@asco.org to confirm the HER2 arm is open and which Northeast site is enrolling.
- 3. Treat this as the low-toxicity HER2 fallback; it has no cytotoxic payload and no neuropathy or ILD liability.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT02623535		recruiting	unconfirmed	Pam Mangat, MS; tapur@asco.org ; www.tapur.org

Manufacturer / sponsor contact

- **Company:** Genentech (Roche); multiple trastuzumab biosimilar makers
- **Notes:** TAPUR supplies drug on-protocol, which sidesteps the off-label payer problem entirely.

Payer / coverage: Enrollment through TAPUR removes drug cost; off-label biosimilar trastuzumab outside the trial is the cheapest HER2 option but still needs prior authorization.

12. zanidatamab (HER2 biparatopic bispecific antibody)

Aliases: ZW25, Ziihera, zanidatamab-hrii

Modality: bispecific_other · **Regulatory:** FDA accelerated approval (2024-11) for previously treated HER2-positive (IHC 3+) biliary tract cancer; investigational in urothelial carcinoma · **Guidelines:** Off-guideline for urothelial carcinoma · **Verified:** 2026-08-20

Zanidatamab is FDA-approved for HER2 IHC 3+ biliary tract cancer, so a treating oncologist can prescribe it off-label for HER2 3+ urothelial carcinoma; that is the realistic path, because the pan-tumor trial cohort that fits this histology bars prior HER2-directed therapy and current T-DXd forecloses it. Off-label use stands or falls on the pending repeat HER2 IHC: the label population is IHC 3+, and a downgrade would gut both the payer case and the biology case.

Next steps

- 1. Wait for the 2026-08 repeat HER2 IHC tier; only a confirmed 3+ supports this row.
- 2. If 3+, have the treating oncologist call JazzCares (1-833-533-5299) about off-label access and patient assistance.
- 3. Position zanidatamab as a post-T-DXd HER2 option rather than a competitor to it; sequencing evidence in that direction exists in breast and gastric cohorts.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06625845		recruiting	no	Clinical Trial Disclosure & Transparency; 215-832-3750

Manufacturer / sponsor contact

- **Company:** Jazz Pharmaceuticals
- **Med info phone:** 1-833-533-5299
- **Product info:** <https://www.ziiherahcp.com/>
- **Notes:** JazzCares line, Monday-Friday 8am-8pm ET.

Payer / coverage: Off-label use in a non-biliary tumor will need prior authorization with the IHC 3+ documentation attached; JazzCares handles coverage casework.

Clinical trial (31)

13. A2B694 (logic-gated Tmod mesothelin CAR-T)

Aliases: A2B694, EVEREST-2, BASECAMP-1

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Entry runs only through the BASECAMP-1 screening study and the whole design depends on tumor HLA-A*02 loss of heterozygosity, which is unmeasured here but computable from the existing exome data at no tissue cost. An ILD-with-steroids history inside a year is an exclusion, so the chest CT reconciliation matters here too. Low probability, but the LOH analysis is free and settles it; NYU is a site.

Next steps

1. Run the HLA-A*02 LOH computation on the banked WES data; it costs nothing and is the gating fact.
2. If LOH is present, ask the sponsor whether BASECAMP-1 will take an off-list histology.
3. Get the ILD question adjudicated; a steroid-requiring ILD history within a year excludes.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06031295		recruiting	unconfirmed	Clinical Trials; ClinicalTrials@a2bio.com; 310-431-9180

Manufacturer / sponsor contact

- **Company:** A2 Biotherapeutics
- **Med info phone:** 310-431-9180

- **Med info email:**ClinicalTrials@a2bio.com
- **Notes:** Contact block copied from the registry record.

14. ART0380 (ATR inhibitor)

Aliases: ART0380

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Part A1 takes any refractory advanced solid tumor, so the ARID1A hypothesis can be tested without a selected cohort, and Northwell, Penn and Jefferson are among the sites. The honest framing: the expansion cohorts select for ATM loss or HRD rather than ARID1A, the synthetic-lethality argument is preclinical, and archival tissue is required against a scarce-tissue case. The pending NGS retrieval could upgrade this row if a homologous-recombination finding surfaces.

Next steps

1. Retrieve the full NGS report first; an HRD or ATM finding changes which cohort applies.
2. Ask the Sarah Cannon line about A1 availability and the archival-tissue minimum.
3. Rank behind the antigen-directed rows; the mechanistic case is hypothesis-grade.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04637268		recruiting	unconfirmed	Sarah Cannon Development Innovations; SCRI.InnovationsMedical@scri.com ; 844-710-6157

Manufacturer / sponsor contact

- **Company:** Artios Pharma
- **Med info phone:**844-710-6157
- **Med info email:**SCRI.InnovationsMedical@scri.com
- **Notes:** Contact block copied from the registry record (Sarah Cannon runs trial operations).

15. ASP2998 (TROP2-directed STING agonist conjugate)

Aliases: ASP2998

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Urothelial carcinoma is a named histology with no cap on prior regimens, and prior TROP2 or topoisomerase-payload exposure is permitted, so the T-DXd history does not close it. A STING payload is a different mechanism from a cytotoxic ADC and may depend less on expression level, which suits a weak TROP2 stain. Hackensack and Long Island sites put it in easy reach; decline the enfortumab vedotin combination arms

given the neuropathy.

Next steps

1. Contact the Hackensack site about escalation-cohort openings for urothelial carcinoma.
2. State the arm preference (monotherapy or pembrolizumab combination, not EV) at first contact.
3. Standard gates apply: T-DXd washout and the measurable-disease re-read.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07281729	2	recruiting	likely	Astellas Pharma Global Development, Inc.; Astellas.registration@astellas.com; 800-888-7704

Manufacturer / sponsor contact

- **Company:** Astellas Pharma
- **Med info phone:**800-888-7704
- **Med info email:**Astellas.registration@astellas.com
- **Notes:** Contact block copied from the registry record; general Astellas medical information is 1-800-727-7003.

16. AVZO-103 (Nectin-4 x TROP2 bispecific ADC)

Aliases: AVZO-103, BEACON-1

Modality: ADC · **Regulatory:** Investigational; FDA Fast Track for post-EV urothelial carcinoma · **Verified:** 2026-08-20

One agent covering both the Nectin-4 and TROP2 features, built for heterogeneous or partly lost antigen, with Fast Track in post-EV urothelial cancer and sites in New Haven, Boston and New York. The load-bearing exclusion is a history of drug-induced ILD: until the conflicting 2026-03 outside CT read is reconciled against the clean 2026-06 study, screening is at risk, and current T-DXd exposure sharpens the question.

Next steps

1. Get a pulmonology adjudication of the 2026-03 versus 2026-06 chest CT reads documented in the chart first.
2. Contact the sponsor about the New Haven or New York site and the current escalation cohort.
3. Plan around the T-DXd washout and the measurable-disease re-read, same as the other post-EV ADC options.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** Avenzo Therapeutics
- **Med info phone:** (858) 239-2944
- **Med info email:** ClinicalTrials@avenzotx.com
- **Notes:** Contact block copied from the registry record.

17. CAdVEC oncolytic adenovirus plus autologous HER2 CAR virus-specific T cells

Aliases: CAdVEC, HER2.CAR AdvST, VISTA

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Geographic scope:** Single US site (Houston, Texas); patient travels for care, so distance is a burden rather than a bar · **Verified:** 2026-08-20

The HER2 bar is IHC 2+ rather than 3+, which keeps this reachable even if the repeat stain downgrades, and bladder cancer is named in the basket. The practical constraints are anatomical and geographic: a lesion must be accessible for intratumoral injection, which a single post-SBRT retroperitoneal node makes awkward, and Houston is the only site.

Next steps

1. Ask the Baylor team whether a post-SBRT para-aortic node is injectable on their protocol before anything else.
2. Keep as the HER2 fallback that survives a 2+ downgrade.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03740256		recruiting	unconfirmed	Shalini Makawita, MD; Shalini.Makawita@bcm.edu ; 832-957-6500

18. CLSP-1025 (TP53 R175H x HLA-A*02:01 T-cell engager)

Aliases: CLSP-1025, GUARDIAN-101

Modality: bispecific_other · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Both molecular gates are already answered in the record: HLA-A*02:01 on 2025-01 typing and TP53 R175H on 2025-06 plasma. An off-the-shelf engager needs no manufacturing slot, four of the sites (NYU, MSK, Hackensack, Johns Hopkins) sit inside the patient's care orbit, and histology is open. The honest caveat is that the R175H call rests on a single 0.09% VAF plasma read, so the sponsor's central confirmation is the real gate and a tissue-level

check would firm the case first.

Next steps

- 1. Confirm R175H in tissue: check whether the 2023 NGS panel's TP53 call specifies R175H before relying on the 0.09% plasma read.
- 2. Contact the sponsor about screening at NYU or MSK and ask what specimen their central mutation confirmation needs.
- 3. Time screening against the T-DXd washout; an engager protocol will want prior-therapy resolution.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06778863		recruiting	likely	Lauren Harshman, MD; LHarshman@clasptx.com; +1-617-812-1431

Manufacturer / sponsor contact

- **Company:** Clasp Therapeutics
- **Med info phone:**+1-617-812-1431
- **Med info email:**LHarshman@clasptx.com
- **Notes:** Contact block copied from the registry record.

19. CRB-701 (site-specific Nectin-4 ADC, MMAE payload)

Aliases: CRB-701, SYS6002

Modality: ADC · Regulatory: Investigational; no approval anywhere · Verified: 2026-08-20

Three exclusions converge on this patient: grade 2 or worse baseline neuropathy, ILD or pneumonitis within 6 months, and a history of other solid tumors, which the 2020 prostate primary may trigger. The lower free-MMAE design is meant to spare nerves, but it is still MMAE in a patient with established CIPN. Reachable only if formal neuropathy grading comes back below 2 and the ILD read resolves clean.

Next steps

- 1. Only pursue if a formal CIPN assessment grades below 2; otherwise the MMAE rows stay closed.
- 2. Ask the sponsor whether a remote, definitively treated prostate primary triggers the second-malignancy exclusion.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06281227		recruiting	unconfirmed	Ian Hodgson, PhD; Clinical@Corbuspharma.com; +1 (617) 963-0105

Manufacturer / sponsor contact

- **Company:** Corbus Pharmaceuticals
 - **Med info phone:** +1 (617) 963-0105
 - **Med info email:** Clinical@Corbuspharma.com
 - **Notes:** Contact block copied from the registry record.
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20. CT-202 (Nectin-4-directed bispecific antibody)

Aliases: CT-202

Modality: bispecific_other · **Regulatory:** Investigational; no approval anywhere · **Geographic scope:** Australia only; sponsor is US-based, so a US-site question is worth asking · **Verified:** 2026-08-20

A Nectin-4 bispecific with no cytotoxic payload at all, which is the cleanest way around the MMAE neuropathy problem, and urothelial cancer is one of three enrolling histologies with ECOG up to 2. It runs only in Australia. The patient's demonstrated willingness to travel internationally keeps it on the list, but enrollment abroad means months on site, and the posted criteria leave the Nectin-4 cutoff and prior-ADC rules unstated.

Next steps

1. Email the sponsor about US site plans; enrolling a US patient in Australia for a phase 1 is a last resort.
2. Ask for the Nectin-4 threshold and prior-ADC rules, which the registry leaves blank.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07545122		recruiting	unconfirmed	Context Clinical Development; 267.225.7416

Manufacturer / sponsor contact

- **Company:** Context Therapeutics
 - **Med info phone:** 267.225.7416
 - **Med info email:** Cntx-ct202-101@contexttherapeutics.com
 - **Notes:** Contact block copied from the registry record; Context is US-based even though the trial enrolls in Australia.
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21. CT-95 (mesothelin x CD3 bispecific antibody)

Aliases: CT-95

Modality: bispecific_other · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Entry is written around mesothelin expression rather than histology, which is the only way a mesothelin-positive

urothelial carcinoma gets a hearing, and Hackensack and Penn are enrolling. Everything waits on a validated mesothelin IHC, because the strong 2026-08 signal is research-grade MXIF and no protocol screens on that. An off-the-shelf antibody also avoids the manufacturing and lymphodepletion cost of the CAR-T alternatives.

Next steps

- 1. Order a validated mesothelin IHC on existing tissue first; mind the 5-10% tumor-content caveat on the nodal cores.
- 2. If positive, contact the Hackensack site and ask which escalation cohort is open and whether they accept outside IHC.
- 3. Measure soluble mesothelin at the same time; an antigen sink changes the dosing conversation.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06736035		recruiting	unconfirmed	Context Clinical Development; 267-225-7416

Manufacturer / sponsor contact

- **Company:** Context Therapeutics
- **Med info phone:**267-225-7416
- **Med info email:**cntx-ct95-101@contexttherapeutics.com
- **Notes:** Contact block copied from the registry record.

22. DS-3939a (TA-MUC1-directed ADC)

Aliases: DS-3939a

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The one enrolling clinical route onto MUC1: a Daiichi Sankyo first-in-human ADC against tumor-associated MUC1 with a DXd payload, verified recruiting with Rhode Island Hospital as the nearest US site. Urothelial fit and the TA-MUC1 threshold need direct sponsor confirmation, the strong MXIF signal still needs a validated stain, and the DXd payload drags the ILD question along with it.

Next steps

- 1. Email CTRinfo@dsi.com to ask whether urothelial carcinoma can screen and what MUC1 assay gates entry.
- 2. Get a validated MUC1 IHC (clone matched to the trial assay) before committing tissue.
- 3. Weigh the DXd payload against the unresolved ILD read; same class as T-DXd.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** Daiichi Sankyo (co-developed with AstraZeneca)
- **Med info phone:** 1-877-437-7763
- **Product info:** <https://dsimedinfo.com/>
- **Notes:** US medical information line, Monday-Friday 9am-6pm ET.

Notes: This trial is not in trials.jsonl; it was found during the access pass as the enrolling study for the surveyed TA-MUC1 ADC. Flagged for a trial_screener re-run rather than added to that file.

23. E303 / SBE303 (Nectin-4 ADC)

Aliases: E303, SBE303

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Entry asks only for an advanced solid tumor that progressed on or could not tolerate standard therapy, with no posted Nectin-4 cutoff and no prior-ADC exclusion, which makes it one of the few Nectin-4 protocols a post-EV patient can approach without a waiver. The registry does not name the payload; if it is MMAE the neuropathy veto applies. US sites are Arizona and Virginia only.

Next steps

1. Email the sponsor to ask what the payload is; that answer decides whether this row is live.
2. If non-MMAE, ask the Fairfax site about slots and the washout requirement.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07524348		recruiting	unconfirmed	Samsung Bioepis; bioepisinfo@samsung.com; +82-32-728-0114

Manufacturer / sponsor contact

- **Company:** Samsung Bioepis
- **Med info phone:** +82-32-728-0114
- **Med info email:** bioepisinfo@samsung.com
- **Notes:** Contact block copied from the registry record; the listed line is the Korean headquarters.

24. EB-DT-NK-UC101 (Nectin-4/HER2 dual-target CAR-NK)

Aliases: EB-DT-NK-UC101, DUET-UC-NK

Modality: CAR-NK · **Regulatory:** Investigational; no approval anywhere · **Geographic scope:** Mainland China only · **Verified:** 2026-08-20

The only protocol found anywhere that explicitly allows both prior enfortumab vedotin and prior HER2-directed therapy, with a Nectin-4 bar low enough to survive partial antigen loss. It enrolls only in China and strongly prefers a fresh post-treatment biopsy, which collides with this patient's tissue scarcity. Logged so the board can see the design exists; not a practical route today.

Next steps

1. Track for a US or ex-China site opening; no outreach warranted now.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07492628		recruiting	unconfirmed	Seni S Lu, PhD; Seni-Lu@beijing-biotech.com; +86 13076790030

25. FAP-targeted radioligand therapy (FAP-2286 / FAPI class)

Aliases: FAP-2286, 177Lu-FAP-2286, LuMIERE, FAPI-46

Modality: radioligand · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

LuMIERE selects on FAP-2286 PET uptake rather than histology, which is the right gate for a stromal target, and Columbia, MSK and UPMC are enrolling. The problem is that the only FAP scan on record argued the other way: avidity in BCG-granulomatous prostate tissue and none in the urothelial disease, with post-SBRT fibroblast activation now muddying any repeat. Retrieve the 2026-06 FAP imaging read before spending a screening slot. The Lilly FIREBOLT study accepts imaging evidence of FAP but its histology list excludes urothelial carcinoma.

Next steps

1. Retrieve the 2026-06 repeat FAP imaging read; it decides whether this axis is real.
2. If FAP-avid disease is shown, ask the Columbia or MSK LuMIERE team to screen with a 68Ga-FAP-2286 PET.
3. Treat granulomatous inflammation and post-SBRT change as expected false-positive sources in any read.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04932510		recruiting	unconfirmed	Novartis Pharmaceuticals; novartis.email@novartis.com; 1-888-689-6682

NCT07213791	recruiting	no	Trial questions or participation questions: 1-877-CTLILLY (1-877-285-4559) or; LillyTrials@Lilly.com; 1-317-615-4559
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Manufacturer / sponsor contact

- **Company:** Novartis
- **Med info phone:** 1-888-689-6682
- **Med info email:** novartis.email@novartis.com
- **Notes:** Sponsor contact as published on the LuMIERE registry record.

26. FT825 / ONO-8250 (off-the-shelf iPSC-derived HER2 CAR-T)

Aliases: FT825, ONO-8250

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

An allogeneic HER2 CAR-T with no apheresis and no manufacturing wait, enrolling on HER2 expression rather than histology, with Yale, MSK and Jefferson among the sites and no exclusion for prior HER2-directed therapy. The gates are the pending contemporary HER2 tier and the trial's own exclusion of prior cell therapy, which this patient clears as long as the TIL and NK programs stay in the discussion stage. One of the few HER2 doors T-DXd exposure does not close.

Next steps

1. Send the trial contact the HER2 history and the 2026-08 re-stain when it lands; ask which dose cohort is open.
2. Confirm the mRNA neoantigen vaccine series does not count against the prior-gene-therapy exclusion before screening.
3. Sequence this after the T-DXd stop decision; no washout conversation is possible while dosing continues.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06241456		recruiting	likely	Fate Trial Disclosure; 858-875-1800

Manufacturer / sponsor contact

- **Company:** Fate Therapeutics
- **Med info phone:** 858-875-1800
- **Med info email:** FateTrialDisclosure@fatetherapeutics.com
- **Notes:** Contact block copied from the registry record.

27. IMA203 / IMA203CD8 (PRAME-directed TCR-T)

Aliases: IMA203, IMA203CD8, ACTengine

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The patient already clears the HLA-A*02:01 gate and MGH, MSK, Penn and three other Northeast sites are enrolling, with a rare-cancer extension cohort giving urothelial carcinoma a route in. The open question is PRAME expression: the trial screens with its own RT-qPCR assay, so the 2026-08 PRAME imaging read informs but does not substitute. Banked apheresis shortens the manufacturing runway if screening succeeds.

Next steps

1. Retrieve the 2026-08 PRAME imaging read; if it suggests expression, contact Immatics about rare-cancer cohort screening.
2. Ask whether the sponsor's PRAME RT-qPCR can run on existing banked tissue given the 5-10% tumor-content caveat.
3. Confirm measurable disease and cell-therapy sequencing against the ongoing vaccine series.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03688024		recruiting	unconfirmed	Immatics US, Inc.; ctgovinquiries@immatics.com ; +1 346 204-5400

Manufacturer / sponsor contact

- **Company:** Immatics
- **Med info phone:** +1 346 204-5400
- **Med info email:** ctgovinquiries@immatics.com
- **Notes:** Contact block copied from the registry record.

28. IMC-P115C (half-life-extended PRAME ImmTAC)

Aliases: IMC-P115C

Modality: bispecific_other · **Regulatory:** Investigational; no approval anywhere · **Geographic scope:** Australia and EU only; no US site in the current record · **Verified:** 2026-08-20

Eligibility is exactly this patient's pair of gates, HLA-A*02:01 plus PRAME positivity, with no histology restriction, but the trial runs only in Australia, France, Italy and Spain. The patient's demonstrated willingness to travel keeps it listed, and Immunocore's medical information line is the right place to ask about US site plans.

Next steps

1. Ask Immunocore medical information whether a US site is planned and whether tebentafusp-style expanded access is contemplated for PRAME.

2. Settle PRAME expression locally first; without it the travel question is moot.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07136136		recruiting	unconfirmed	Immunocore Medical Information Global; 844-466-8661

Manufacturer / sponsor contact

- **Company:** Immunocore
- **Med info phone:** 844-466-8661
- **Med info email:** medical.information@immunocore.com
- **Notes:** Global medical information contact, as published on the registry record.

29. LCB84 (TROP2-directed ADC)

Aliases: LCB84

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Dose escalation takes any refractory solid tumor and permits prior TROP2-directed therapy, so the weak stain and the T-DXd history do not close it. The problem is protocol biopsies: mandatory pre- and on-treatment sampling in the enrichment cohorts, against a tumor with one fresh-frozen vial left and nodal cores running 5 to 10% tumor content. Dana-Farber is the nearest site.

Next steps

1. Ask the sponsor whether an escalation slot without mandatory biopsies is open before spending a screening visit.
2. Prefer ASP2998 or the TROPHY-U-01 route if biopsy demands cannot be reduced; tissue is the scarcest resource in this case.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05941207		recruiting	unconfirmed	David Browning; dbrowning@ligachembio.com ; +1-615-975-7776

Manufacturer / sponsor contact

- **Company:** LigaChem Biosciences
- **Med info phone:** +1-615-975-7776
- **Med info email:** dbrowning@ligachembio.com

- **Notes:** Contact block copied from the registry record.

30. LY4052031 (Nectin-4 ADC, camptothecin payload)

Aliases: LY4052031, NEXUS-01

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Cohort B2 of NEXUS-01 is written for exactly this history: urothelial carcinoma, prior enfortumab vedotin required rather than excluded, and a topoisomerase payload that sidesteps the MMAE neuropathy. Early data put the post-EV response rate near 47%. Six Northeast sites including MSK, Mount Sinai, Columbia, NYU and MGH. The two screening gates are a measurable RECIST lesion after SBRT to the only known nodal site and the washout from T-DXd once a stop decision is made.

Next steps

1. Call the Lilly trials line or have the treating team contact the nearest site (MSK or Mount Sinai) to confirm B2 slot availability.
2. Get a baseline CT re-read to establish whether a measurable, non-irradiated target lesion exists after the 2026-06 SBRT.
3. Confirm the required washout from T-DXd; both drugs carry topoisomerase payloads and the site may also want the ILD discrepancy resolved.
4. Flag the sequential topoisomerase-I exposure question (deruxtecan then camptothecin) with the investigator; cross-resistance is unstudied.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06405069		recruiting	likely	Trial questions or participation questions: 1-877-CTLILLY (1-877-285-4559) or; LillyTrials@Lilly.com; 1-317-615-4559

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company
- **Med info phone:** 1-317-615-4559
- **Med info email:** LillyTrials@Lilly.com
- **Notes:** Lilly clinical-trials line (1-877-CTLILLY), as published on the registry records.

31. NEOK002 (EGFR x MUC1 bispecific ADC)

Aliases: NEOK002

Modality: ADC · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The deciding exclusion is procedural: the most recent systemic therapy cannot have been a topoisomerase-I-payload ADC, which is exactly what T-DXd is, so the door only opens after an intervening regimen. A known ILD history is also excluded. MUC1 remains a research-grade signal without a validated stain, so this row is parked behind both a sequencing problem and an assay problem.

Next steps

1. Park until an intervening non-topo-I regimen exists; then revisit with a validated MUC1 stain and the ILD read.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07612189		recruiting	no	Beth Lepping, RN, BSN, MS; blepping@neokbio.com ; 610-207-2119

Manufacturer / sponsor contact

- **Company:** NEOK Bio
- **Med info phone:** 610-207-2119
- **Med info email:** blepping@neokbio.com
- **Notes:** Contact block copied from the registry record.

32. NT-175 (autologous TCR-T targeting TP53 R175H)

Aliases: NT-175, Neogene TP53 R175H TCR-T

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The histology list ends in any other solid tumor, so entry rides on the two molecular gates the patient meets on paper, and banked apheresis shortens the manufacturing runway. Prior adoptive cell or gene therapy is excluded, which currently reads clean but makes the ongoing mRNA vaccine series something to disclose rather than assume benign. Boston, New Brunswick, New York and Pittsburgh sites; AstraZeneca runs the program through Neogene.

Next steps

1. Call the AstraZeneca study information center to ask about slot and manufacturing timelines at the New York or Boston site.
2. Disclose the vaccine series and confirm it does not trip the gene-therapy exclusion.
3. Run the same tissue-level R175H confirmation as the CLSP-1025 row; one confirmation serves both.
4. Compare timelines: the engager (CLSP-1025) doses off the shelf while this needs manufacture; sequencing one after the other is worth asking both sponsors about.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com
- **Notes:** AstraZeneca Clinical Study Information Center, as published on the registry records.

33. SynKIR-110 (mesothelin KIR-CAR T cells)

Aliases: SynKIR-110

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The histology list is closed to ovarian, peritoneal, fallopian tube, cholangiocarcinoma and mesothelioma, so this patient cannot enroll. The row exists because the KIR-CAR architecture is a distinct attempt at the persistence problem that has limited mesothelin CAR-T, and Penn is local to the case's orbit if a basket ever opens.

Next steps

1. No action; watch for a histology expansion.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05568680		recruiting	no	—

34. TNhYP218 (anti-mesothelin TNaive/SCM CAR T cells)

Aliases: TNhYP218, hYP218

Modality: CAR-T · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

NCI dose escalation takes mesothelin-expressing solid tumors generally with no limit on prior regimens, and the trial runs its own mesothelin assay at screening, so a local validated stain qualifies the referral rather than deciding it. Measurable disease is required and Bethesda is the only site. The banked apheresis does not shortcut anything here since the protocol collects its own product, but this patient's logistics make an NIH referral routine.

Next steps

1. Send pathology and the MXIF report to the NCI contacts and ask whether they will run their mesothelin assay on existing tissue.

2. Confirm the measurable-disease read; NIH screening will want a qualifying target lesion.
3. Coordinate timing with the vaccine series and any T-DXd washout.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06885697		recruiting	unconfirmed	Maria Gracia L Agra, R.N.; mariagracia.agra@nih.gov; (240) 858-3152

35. XYA02 (MUC1-directed agent)

Aliases: XYA02

Modality: other · **Regulatory:** Investigational; no approval anywhere · **Geographic scope:** Australia only, not yet open · **Verified:** 2026-08-20

Not yet open, Australia only, and the registry does not say what the construct is or which MUC1 epitope it hits, which matters because normal epithelium carries wild-type MUC1. Track rather than pursue.

Next steps

1. Track for opening and for construct disclosure; no outreach warranted now.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07670212		not yet recruiting	unconfirmed	Tiffany Sepp; clinicaltrial@xyonetx.com; 617-710-0770

36. [225Ac]Ac-AKY-1189 (Nectin-4 actinium-225 radioligand)

Aliases: AKY-1189, [64Cu]Cu-AKY-1189

Modality: radioligand · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The one Nectin-4 re-targeting route that does not depend on retained uniform antigen expression or an MMAE payload: prior EV is explicitly permitted, the alpha emitter kills by crossfire, and the 64Cu imaging step answers the current-expression question that the 2026-06 Nectin-4 scan left open. Mount Sinai is an enrolling site. Gates are a measurable lesion after SBRT and a roughly 3-week washout from T-DXd.

Next steps

1. Email the sponsor inquiry line to ask about screening at Mount Sinai and current dose-escalation stage.

2. Confirm measurable disease post-SBRT with the site; nodal-only disease after radiation is the recurring gate.
3. Time screening against the T-DXd washout; the imaging visit can be scheduled while that clock runs.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07020117		recruiting	likely	Janet Horton, MD; 978-208-3986

Manufacturer / sponsor contact

- **Company:** Aktis Oncology
- **Med info phone:** 978-208-3986
- **Med info email:** AKY-1189-01inquiries@aktisoncology.com
- **Notes:** Contact block copied from the registry record.

37. ceralasertib (AZD6738, ATR inhibitor)

Aliases: AZD6738

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Ceralasertib remains investigational and none of its recruiting studies fit: the active combinations run in lung cancer, melanoma and gynecologic cohorts, and the ARID1A-relevant academic studies have closed. The dossier's ceralasertib evidence supports the ATR hypothesis, but the enrollable embodiment of that hypothesis for this patient is ART0380's all-comers arm, not this drug.

Next steps

1. Route ATR-axis interest through the ART0380 row; no ceralasertib protocol currently fits this case.

Manufacturer / sponsor contact

- **Company:** AstraZeneca
- **Med info phone:** 1-877-240-9479
- **Med info email:** information.center@astrazeneca.com
- **Notes:** AstraZeneca Clinical Study Information Center, as published on the registry records.

38. disitamab vedotin (RC48-ADC)

Aliases: RC48, RC48-ADC, DV, Aidixi

Modality: ADC · **Regulatory:** NMPA-approved (China) for HER2-overexpressing urothelial and gastric cancer; investigational in the US (no FDA approval) · **Geographic scope:** Approved and prescribable in mainland China only; US access is trial-only and the trial excludes this patient · **Verified:** 2026-08-20

Every cohort of the US registration trial is closed to this patient: prior HER2-directed therapy (T-DXd), prior MMAE ADC (enfortumab vedotin), and baseline grade 2+ neuropathy are each disqualifying, and cohorts A and B also require a prior platinum line he never had. The drug is approved only in China, so there is no US off-label route either. Logged as closed so nobody spends time on it.

Next steps

- 1. No action; revisit only if a next-generation disitamab protocol without the prior-HER2/prior-MMAE exclusions opens.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04829329		recruiting	no	Pfizer CT.gov Call Center; 1-800-718-1021

Manufacturer / sponsor contact

- **Company:** Pfizer (Seagen); originator RemeGen (China)
- **Med info phone:**1-800-718-1021
- **Med info email:**ClinicalTrials.gov_Inquiries@pfizer.com
- **Notes:** Pfizer clinical-trials call center, as published on the registry record.

39. neoantigen-selected TIL with aldesleukin (NCI Surgery Branch)

Aliases: NCI TIL, young TIL

Modality: other · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Genitourinary tumors are named and the selection principle matches the neoantigen program this patient is already running, with 24 predicted neoepitopes and serial PBMCs banked. The binding constraint is surgical: TIL generation needs a lesion resectable with minimal morbidity, and the only known disease is a post-SBRT para-aortic node. High-dose aldesleukin tolerance is the other conversation. The family's NCI referral costs one phone call to the same Bethesda center as the TNhYP218 row.

Next steps

- 1. Call the Surgery Branch recruitment line with imaging and ask whether any current or anticipated lesion supports TIL harvest.
- 2. Offer the banked fresh-frozen tumor vial and serial PBMCs; the branch decides what it can use.
- 3. Revisit at the next progression event; a new unirradiated lesion changes this from marginal to live.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
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40. personalized neoantigen vaccines (peptide + mRNA programs)

Aliases: neoantigen peptide vaccine, mRNA neoantigen vaccine

Modality: vaccine · **Regulatory:** Investigational; administered under the programs' own protocols · **Verified:** 2026-08-20

The patient is already receiving this class through two investigational programs, with the mRNA series ongoing and the next EliSpot the decision point, so access is not the question. No registered vaccine trial adds anything: the one in the dossier is triple-negative breast only. The practical access work is keeping the current programs supplied and pairing the vaccine question with the checkpoint-intensification decision the board is weighing.

Next steps

1. Complete the 2026-09 mRNA dose and the post-series EliSpot; that readout drives the whole vaccine/ACT branch.
2. If immunogenicity stays CD4-skewed and modest, redirect energy to the TIL and TCR rows rather than a third vaccine program.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT03626967		recruiting	no	—

41. sacituzumab tirumotecan (sac-TMT, MK-2870)

Aliases: sac-TMT, MK-2870, SKB264

Modality: ADC · **Regulatory:** Investigational in the US (approved in China for some indications); no FDA approval · **Verified:** 2026-08-20

The phase 3 requires prior platinum, prior anti-PD-(L)1 and prior EV, and this patient never received platinum, so the trial is closed on a line-of-therapy technicality rather than biology. The control arm is a taxane or vinflunine, which the neuropathy would veto anyway. Kept visible because the platinum gap recurs across the registered post-EV landscape and is worth a strategic conversation with the treating team.

Next steps

1. No enrollment path as written; ask MSK whether other sac-TMT urothelial protocols without the platinum requirement are planned.
2. Log the strategic question: never having received platinum forecloses a growing set of trials, and whether to ever give platinum is a treatment-planning decision, not a screening one.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT07439295		recruiting	no	Toll Free Number; Trialsites@msd.com; 1-888-577-8839

Manufacturer / sponsor contact

- **Company:** Merck Sharp & Dohme
- **Notes:** Trial-site inquiries go through the registry contact on each study record; no separate medical-information line verified this pass.

42. tuvusertib (M1774, ATR inhibitor) with avelumab

Aliases: M1774, avelumab, Bavencio

Modality: small_molecule · **Regulatory:** Tuvusertib investigational; avelumab FDA-approved (urothelial maintenance among others) · **Verified:** 2026-08-20

The one trial that selects on ARID1A loss of function pairs tuvusertib with avelumab, but it enrolls endometrial cancer only, so this patient cannot join. It stays in the file as the reference point for how the ARID1A-ATR-plus-checkpoint idea is being tested, and as the design the immune-intensification branch here most resembles.

Next steps

1. Ask the DFCI investigator whether an ARID1A basket or urothelial arm is planned; otherwise no action.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT06528564		recruiting	no	Panagiotis Konstantinopoulos, MD, PhD; 617-632-5269

43. vepugratinib (LOXO-435, FGFR3 inhibitor)

Aliases: LY3866288, LOXO-435, FORAGER-1

Modality: small_molecule · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

Cohort B7 pairs a qualifying FGFR3 alteration with HER2 expression, the one place in the registry where this patient's HER2 result and his unread FGFR3 status intersect. Everything rides on the sequencing retrieval: FGFR3 is unknown in the derived record and roughly 15 to 20% of metastatic urothelial carcinoma qualifies. Eight-plus

Northeast sites including MSK, NYU, Cornell, Columbia and MGH make this instantly actionable if the report shows an alteration.

Next steps

1. Retrieve the full tissue NGS report and read out FGFR3; this is already the case's top data-retrieval item.
2. If altered, contact the Lilly trials line about cohort B7 at MSK or NYU.
3. If FGFR3 is altered, also weigh approved-erdafitinib off-label logic against this trial with the treating team.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT05614739		recruiting	unconfirmed	Trial questions or participation questions: 1-877-CTLILLY (1-877-285-4559) or; LillyTrials@Lilly.com; 1-317-615-4559

Manufacturer / sponsor contact

- **Company:** Eli Lilly and Company
- **Med info phone:** 1-317-615-4559
- **Med info email:** LillyTrials@Lilly.com
- **Notes:** Lilly clinical-trials line (1-877-CTLILLY), as published on the registry records.

Notes: Erdafitinib itself is FDA-approved for FGFR3-altered mUC post-chemotherapy; if FGFR3 comes back positive, that on-label option should be screened by the trial_screener as a follow-up, since it is not in the current dossier.

Not yet accessible (1)

44. brenetafusp (IMC-F106C, PRAME ImmTAC)

Aliases: IMC-F106C, PRISM-MEL

Modality: bispecific_other · **Regulatory:** Investigational; no approval anywhere · **Verified:** 2026-08-20

The parent phase 1/2 stopped enrolling and the only new brenetafusp study is an NCI sarcoma protocol that has not opened and would not take urothelial carcinoma anyway. Nothing to enroll in today; the class's open doors for this patient are IMA203 and, geography permitting, IMC-P115C. If PRAME comes back strongly positive, a compassionate-use conversation with Immunocore is the remaining brenetafusp route.

Next steps

1. If PRAME confirms positive, ask Immunocore medical information about expanded access to brenetafusp for an HLA-matched, PRAME-positive off-melanoma patient.
2. Otherwise route PRAME interest through the IMA203 row.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT04212266	1	active not recruiting	no	—
NCT07626367	2	not yet recruiting	no	—

Manufacturer / sponsor contact

- **Company:** Immunocore
- **Med info phone:** 844-466-8661
- **Med info email:** medical.information@immunocore.com
- **Notes:** Global medical information contact, as published on the registry record.

Unavailable (12)

45. Ad-sig-hMUC1/ecdCD40L adenoviral vaccine

Modality: vaccine · **Regulatory:** Investigational; program dormant · **Verified:** 2026-08-20

The single phase 1 record has unknown status and has not been updated in years; no active program exists. No access path.

Next steps

1. None.

46. ETx-22 (Nectin-4 ADC, exatecan payload, preclinical)

Aliases: ETx-22

Modality: ADC · **Regulatory:** Preclinical; no clinical trial registered · **Verified:** 2026-08-20

ETx-22 has no registry entry and no first-in-human trial; its developer Emergence Therapeutics was acquired by Eli Lilly in 2023 and the asset has not surfaced as a named clinical program. The clinically open way to get a topoisomerase payload onto Nectin-4 today is Lilly's own LY4052031, which has its own row and is enrolling.

Next steps

1. Pursue LY4052031 instead; it is the enrolling embodiment of the same idea.

47. M7A-DC / M7GA-DC (mesothelin DARPIn-MMAE conjugates, preclinical)

Modality: other · **Regulatory:** Preclinical; no clinical trial registered · **Verified:** 2026-08-20

Preclinical constructs with no registered trial, and MMAE payloads would face the neuropathy veto in any case. No access path.

Next steps

1. None.
-

48. MUC1-C inhibition (GO-203 class, preclinical for this use)

Modality: small_molecule · **Regulatory:** Investigational; no enrolling solid-tumor trial · **Verified:** 2026-08-20

Marked considered-excluded upstream and with no enrolling solid-tumor program, MUC1-C inhibitors have no access path for this case.

Next steps

1. None.
-

49. Nectin-4-directed CAR T cells (preclinical)

Modality: CAR-T · **Regulatory:** Preclinical; no IND on record · **Verified:** 2026-08-20

Nectin-4 CAR-T exists only as preclinical work; no clinical program is enrolling anywhere. The concept is on the dossier because antigen-priming data suggest a route around post-EV antigen loss, but there is nothing to call about. The Chinese dual-target CAR-NK row is the closest clinical relative.

Next steps

1. None; revisit if a first-in-human Nectin-4 CAR program posts.
-

50. Nectin-4-targeted near-infrared photoimmunotherapy (preclinical)

Modality: other · **Regulatory:** Preclinical; no clinical trial registered · **Verified:** 2026-08-20

Preclinical only, and the surveyor already marked it considered-excluded. No clinical program, no access path, and deep nodal disease is a poor anatomical fit for a light-activated therapy in any case.

Next steps

1. None.
-

51. R7059-DXd (epitope-distinct TROP2 ADC, preclinical)

Modality: ADC · **Regulatory:** Preclinical; no clinical trial registered · **Verified:** 2026-08-20

Preclinical only and already marked considered-excluded by the surveyor; no registered trial, no access path. The clinically open TROP2 rows (ASP2998, LCB84, Dato-DXd, sacituzumab govitecan) cover the axis.

Next steps

1. None.
-

52. anetumab ravtansine (mesothelin ADC)

Aliases: BAY94-9343

Modality: ADC · **Regulatory:** Investigational; sponsor development discontinued · **Verified:** 2026-08-20

Bayer stopped developing anetumab ravtansine after the phase 2 mesothelioma failure; remaining NCI combination studies are active-not-recruiting and no new enrollment exists. A DM4 payload would also have run into the neuropathy veto. The row closes the loop so the mesothelin-ADC question does not linger.

Next steps

1. None.
-

53. berzosertib (M6620, ATR inhibitor)

Aliases: M6620, VX-970

Modality: small_molecule · **Regulatory:** Investigational; sponsor development deprioritized, no recruiting trial · **Verified:** 2026-08-20

No berzosertib study is recruiting; the NCI combinations are active-not-recruiting and the sponsor deprioritized the program after the 2023-24 readouts. The evidence stays as mechanism support for the ATR axis; there is no way to receive the drug.

Next steps

1. None; the ATR door is ART0380.
-

54. gatipotuzumab (anti-TA-MUC1 antibody)

Aliases: PankoMab-GEX

Modality: monoclonal_antibody · **Regulatory:** Investigational; no active development · **Verified:** 2026-08-20

All gatipotuzumab studies completed years ago and no active development program is registered; the naked-antibody approach did not advance past phase 2. The evidence stays as mechanism support for the TA-MUC1 axis, whose live embodiment is DS-3939a.

Next steps

- 1. None; pursue the DS-3939a row instead.

55. gavocabtagene autoleucel (gavo-cel)

Aliases: gavo-cel, TC-210

Modality: CAR-T · **Regulatory:** Investigational; development deprioritized after the 2023 TCR2/Adaptimmune merger · **Verified:** 2026-08-20

The phase 1/2 stopped enrolling and the program was deprioritized after TCR2 Therapeutics folded into Adaptimmune in 2023; no successor trial is recruiting. The dossier keeps the clinical data as evidence for the mesothelin axis, but there is no way to receive gavo-cel today.

Next steps

- 1. None; the open mesothelin cell-therapy doors are TNhYP218 and, conditionally, A2B694.

Trial pathways

NCT	Phase	Status	Eligible	Central contact
NCT039017252		active not recruiting	no	—

56. huCART-meso (fully human anti-mesothelin CAR T cells)

Aliases: huCART-meso, CART-meso

Modality: CAR-T · **Regulatory:** Investigational; academic program with no recruiting protocol · **Verified:** 2026-08-20

The Penn huCART-meso program has no recruiting study: the monotherapy trials completed or terminated and the remaining combination study is active-not-recruiting in pancreatic and ovarian cancer. The clinical experience stays in the dossier as evidence; enrollment is not possible. TNhYP218 at the NCI is the enrolling mesothelin CAR-T alternative.

Next steps

- 1. None; route mesothelin cell-therapy interest to the TNhYP218 row.